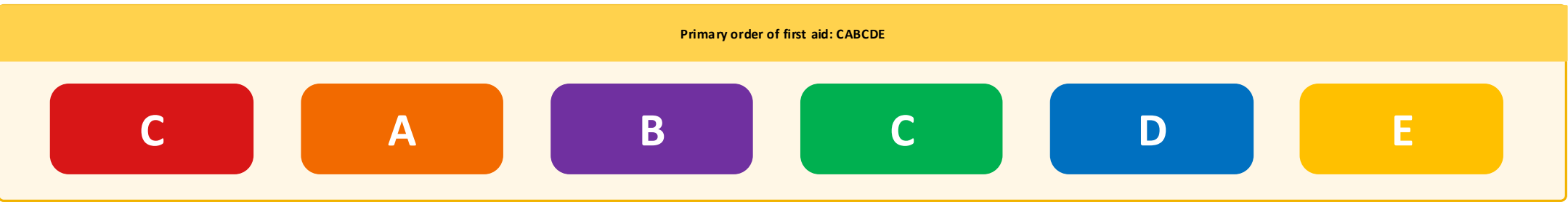


Scene and Safety Management		
Upon arrival • Note time of arrival • Tell control • Call for help or backup early	Check for Safety 1-2-3 1. You - put on gloves/glasses immediately 2. Environment - is there a crowd/other vehicles? 3. Patient - are they being aggressive?	Read the scene • Consider the Mechanism of Injury (MOI) • 90% diagnosis



C - Catastrophic bleeding	Manual In-Line Stabilisation (MILS)	4 points of alignment	Establish level of consciousness
Tourniquet • single loop onto skin 2 inches above wound • double loop (thigh) • continued bleed may need 2 tourniquets	• Stabilize head and neck with: ➢ knees (if possible) ➢ one hand ➢ nearby equipment	1. Nose 2. Sternum 3. “fluffy bits” 4. Eyes looking forward	• Alert • Confused • Verbal • Pain (airway concern) • Unresponsive to pain

A - Airway	Is the airway noisy or silent?		
Noisy = partially obstructed • Bubbling -> check for fluid • Snoring -> tongue blockage	→	• Check hard structures (jaw, cheek bones, teeth) • Check soft tissue (lips, tongue, larynx, nose – milk for signs of bleeding)	→ • Open mouth and clear foreign objects • For blood/vomit, use postural drainage then suction
Silent = complete obstruction • back blows • chest thrusts	→	Create an airway • Head tilt • chin lift	→ - If struggling, consider • x2 NPA (6 Female / 7 Male) • OP tube • Keep going manually

B - Breathing	Expose chest to waistline, if possible.				
Rate – Count the rate of breathing • Goalposts of life 10-30 Volume – Assess the volume of breathing	Oxygen device • free flow or assisted • Ventilation depending on rate • (CD cylinder ≈ 20 min @ 15 L/min)	Feel the chest • Holes on the chest (cover all vent one on each side) • Chest seal to sucking chest wound	Look at the chest • Pattern bruising on the chest (internal problem)	Armpits	Search • Back • Sides • Shoulders • clavicles
				Holes	

C - Circulation	Check for location of blood and pulse.		Check for radial pulse:	No pulse?
If on the floor • Stop all external bleeding • direct pressure • hemostatic gauze to junctional bleeds • elevation and indirect pressure • windlass dressing and splint long bones and pelvis	If on chest or abdomen: • Surgeon needed asap If on pelvis: • Surgeon needed asap • Splint if unstable • Do NOT rock	If on long bones: • femur alignment (hip, knee, 2nd toe) Look for signs of internal bleeding • pattern bruising	• Check both sides • Check the rate • Capillary refill time (CRT) <2s • Check the volume • Check skin colour • pale and clammy	• legs to 45° (only if no femur/pelvis fracture) • preserve the first clot by applying careful pressure

D - Disability	Reassess every 5 min	Check PEARL: Pupils Equal And Reactive to Light	Consider Pentrox – ABACKPAC
• Are they moving all 4 limbs? • Assess disability every 5 mins	• Alert • Verbal • Pain (airway concern) • Unresponsive to pain	• Assess analgesia need • Pain score 1–10	• Questions: check safe with 2nd

E – Exposure/Evacuation	Keep warm, pack, wrap, evacuate		
Do not overexpose patient to element, risk of hypothermia	Keep them warm	Protect the head	Prepare evacuation route

EMERGENCY GUIDES

D13 TRAINING COURSE